

Project Report: Exploring the Changes in Blood Glucose Data and the Impact of Diet on Drug Addicts

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Abstract

This report serves as a detailed documentation of the project, comprehensively summarizing the analysis process and the results of data related to drug addicts. The data sources include Continuous Glucose Monitoring (CGM) blood glucose data, meal records, and psychological assessment forms, among other multidimensional information. This report not only covers each update, but also systematically organizes the overall research progress, relevant experimental results, and preliminary exploratory studies, providing a complete reference for future research and analysis. See the GitHub Repository.

1 Database Introduction

The database was developed as part of a one-year longitudinal follow-up study titled "Personalized Nutrition and Health Program for Individuals in Drug Rehabilitation," a joint initiative by the laboratory at Westlake University and the Zhejiang Provincial Drug Rehabilitation Administration. The study focuses on health management and personalized nutrition interventions for individuals undergoing compulsory drug rehabilitation, aiming to explore the long-term effects of scientifically designed nutritional strategies on their physical and psychological well-being.

During the study, 200 voluntary participants were recruited from three compulsory drug rehabilitation centers in Zhejiang Province. Participants underwent regular and systematic assessments, including baseline health data collection, physiological monitoring, nutritional status assessment, psychological health assessment, and lifestyle surveys. This comprehensive dataset integrates physiological, psychological, and behavioral dimensions, providing a valuable resource for precision health management in drug rehabilitation populations. In addition, the data serve as a scientific basis for future public health policies and clinical intervention strategies.

1.1 Data Collection Process

Participants were required to meet the following inclusion criteria: **age between 18 and 65 years**, admission to the rehabilitation center **within the past 3 months**, and **no** severe organ failure, infectious diseases (such as hepatitis A, hepatitis B, or Human Immunodeficiency Virus (HIV)), severe mental disorders, or cancer. The data collection process was conducted in three phases: Day 1 (D-1), Day 2 (D-2), and Day 14 (D-14).

On **D-1**, participants first filled out registration forms (including name, ID, identification number, sex, etc.) and received numbered tags along with labeled saliva collection kits. They then proceeded to a room to complete the questionnaire survey. The staffs performed anthropometric measurements, including height, weight, waist circumference, hip circumference, neck circumference, upper arm circumference, calf circumference, body composition, and blood pressure (systolic and diastolic). Then each participant was fitted with a CGM device, which was activated for two minutes. Additionally, participants were provided with a sample collection kit, which included a fecal sample cup, an additional 5ml EP tube, and a urine collection device, as well as a dietary diary. The research team also pre-printed dietary records and standardized cafeteria meal logs based on the scheduled meal plan of the rehabilitation center. Finally, participants were instructed to collect fasting urine and stool samples the next morning.

On **D-2**, the staffs collected fasting urine and stool samples, as well as blood samples.

On **D-14**, staffs assisted participants in removing the CGM device and storing it in designated collection bags, while also retrieving dietary diaries and standardized cafeteria meal records.

The cafeteria staff recorded standardized meal weights over a 14-day pe-

riod. The staff pre-printed dietary record sheets based on the meal plan for the following two weeks and distributed them to the cafeteria staff. Before cooking, the staff weighed and recorded the total raw weight of each ingredient used (e.g., rice, vegetables, oil, salt). After cooking, they recorded the total cooked weight and then weighed a standard portion of the cooked meal, documenting the data in the log sheet.

The original study design included two follow-up visits, but **the current database contains only baseline data from the first visit.**

1.2 Key Features

1.3 Potential Applications

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2 State of the Art

xxxxx[1]

3 xxx

3.1 xxx

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3.2 xxx

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References

- [1] W.-H. Jia, H.-D. Qin, Non-viral environmental risk factors for nasopharyngeal carcinoma: a systematic review, in: Seminars in Cancer Biology, Vol. 22, Elsevier, 2012, pp. 117–126.